

After-Hours Telehealth Triage Guidelines Database

Documentation for Microsoft Access Database

SCHMITT-THOMPSON CLINICAL CONTENT

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AFTER-HOURS TELEHEALTH TRIAGE GUIDELINES DATABASE

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Purpose

This document provides a brief description of the After-Hours database. This database stores the Schmitt-Thompson Clinical Content (STCC) adult and pediatric telehealth triage guidelines used by medical call centers providing after-hours telehealth triage.

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The reasons for this request are simple:

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Introduction

Microsoft Office Access is a relational database management system from Microsoft that combines the relational Microsoft Jet Database Engine with a graphical user interface and software-development tools. It is a member of the Microsoft Office suite of applications.

There are different versions of Microsoft Access including Access 2002-2003, Access 2007, and Access 2010, Access 2017.

The databases described in this document is a Microsoft Access 2002-2003 file (<http://support.microsoft.com/ph/2509>).

There are separate databases for the pediatric and adult content. The database structure for these two databases is identical.

XHTML

Some of the clinical content within the database is stored as XHTML. The XHTML content was generated using the XStandard XHTML editor (<http://xstandard.com/>). For software that uses web-enabled controls, the XHTML allows greater control over formatting (e.g., bullets, bold, italics, web-links) and improved readability for the end-user. Specifically, within the ALGORITHM table the following fields contain XHTML:

- *BackgroundXHTML*
- *DefinitionXHTML*
- *FirstAidXHTML*

Corresponding plain text fields are present for these XHTML fields:

- *Background*
- *Definition*
- *FirstAid*

Within the SUPPLEMENTAL table the following fields contain XHTML:

- *Content_XHTML*

A corresponding plain text field is present:

- *Content*

Database Tables

There are 14 tables in the database that store the telehealth triage clinical content.

Table	Description
Acuity	A look-up table that provides a numeric acuity (urgency) score of 1 to 5. The <i>Acuity</i> field in the Algorithm and Disposition tables use this.
Advice	This table contains the Care Advice (CA) for each guideline.
Algorithm	This is the primary table in the database. Each row (record) corresponds to one telehealth triage guideline (i.e., algorithm, protocol). This table contains many metadata fields for sorting and selecting a guideline (e.g., by age, gender at birth). This table contains many content fields such as the guideline Definition, Background Information (BI), and First Aid.
AlgorithmReference	This table provides a one-many-one relationship between Algorithm and Reference .
AlgorithmSearchwords	This table provides a one-many-one relationship between Algorithm and Searchword .
AlgorithmSupplemental	This table provides a one-many-one relationship between Algorithm and Supplemental .
Disposition	This table lists the Dispositions (e.g., EMS 911 Now, Go to ED Now), See PCP Within 24 Hours) to which each Triage Assessment Questions will map.
Question	This table contains the Triage Assessment Questions (TAQs) for each guideline.
QuestionAdvice	This table provides a one-many-one relationship between the Question table to the Advice table.
Reference	This table contains the references (article citations) for each guideline. These references show the evidence-base for a guideline.
Searchword	This table contains all of the search words (metadata; key words).
Supplemental	This table contains supplemental non-guideline content (e.g., list of reviewers, OTC drug dosage tables)
System	A look-up table that is used for sorting, selecting, and reporting on the guidelines.
Type	A look-up table that is used for sorting, selecting, and reporting on the guidelines.

Table – AcuityRating

A look-up table that provides a numeric acuity (urgency) score of 1 to 5. The *Acuity* field in the **Algorithm** and **Disposition** tables use this.

Field Name	Data Type	Description (Optional)
AcuityRating_Numeric	Number	Numeric acuity score: 1 to 5 with 1 being the most serious (urgent)
AcuityRating_Text	Short Text	Text description of acuity score
AcuityRating_Color	Short Text	3-color text description: red, yellow, green
AcuityRating_Bullet	OLE Object	Example bullets consistent with AcuityRating_Color
AcuityRating_Color_Alternate	Short Text	5-color text description: dark red, red, yellow, green, light green
AcuityRating_Color_Alternate_Img	OLE Object	
AcuityRating_Title	Short Text	

Table – Advice

This table contains the Care Advice (CA) for each guideline.

Field Name	Data Type	Description (Optional)
AdviceID	AutoNumber	Unique ID
AlgorithmID	Number	Secondary key; identifies which guideline uses this Care Advice
DateCreated	Date/Time	Date created
LastUpdate	Date/Time	Date last updated
Advice	Long Text	Care advice statement in TEXT
Advice_XHTML	Long Text	Care advice statement in XHTML
PatientHealthInfo	Yes/No	Care advice appropriate for distribution to patient via web portal or email.
AdviceSnap	Short Text	First line of Care Advice Statement; used by Authors; not needed by end-users
AlgorithmOrder	Number	Used by Authors to track equivalent care advice

Here are two examples from adult After-Hours database showing *Advice.AlgorithmOrder* and *Advice.Advice_XHTML* (formatted as XHTML or RTF).

40. Call EMS 911 Now:

- Immediate medical attention is needed. You need to hang up and call 911 (or an ambulance).
- *Triager Discretion:* I'll call you back in a few minutes to be sure you were able to reach them.

1125. Antihistamine Medicines for Moderate to Severe Itching:

- For moderate to severe itching, you can take either cetirizine, fexofenadine, or loratadine.
- They are over-the-counter (OTC) antihistamine medicines. You can buy them at a drugstore or grocery store.
- **Cetirizine (Reactine, Zyrtec):** The adult dose is 10 mg. You take it once a day. Cetirizine is available in the United States as Zyrtec and in Canada as Reactine.
- **Fexofenadine (Allegra):** In the United States, the adult dose is one 24-hour tablet (180 mg) once a day. In Canada, the adult dose is one 24-hour tablet (120 mg) once a day. Or, you can take one 12-hour (60 mg) tablet 2 times a day.
- **Loratadine (Alavert, Claritin):** The adult dose is 10 mg. You take it once a day. Loratadine is available in the United States as Alavert and Claritin; it is available in Canada as Claritin.

Table – Algorithm

This is the primary table in the database. Each row (record) corresponds to one telehealth triage guideline (i.e., algorithm, protocol). This table contains many metadata fields for sorting and selecting a guideline (e.g., by age, gender at birth). This table contains many content fields such as the guideline Definition, Background Information (BI), and First Aid.

Field Name	Data Type	Description (Optional)
AlgorithmID	AutoNumber	Unique ID
Author	Long Text	Author of guideline
Copyright	Long Text	Copyright notice for each guideline
CreateDate	Date/Time	Date guideline created (or first imported into Content Management System database)
LastUpDate	Date/Time	Last date that the guideline record in this table was changed
LastReviewDate	Date/Time	Last date that the guideline record in this table was reviewed
Title	Short Text	Title of Guideline (e.g., Abdominal Pain - Male)
Title_UpperCase	Short Text	Title of Guideline in UPPER CASE
Title_LastUpdate	Date/Time	Last date that the title was changed
Definition	Long Text	Text definition describing this guideline
DefinitionXHTML	Long Text	Definition stored as XHTML
Definition_LastUpdate	Date/Time	Last date that the definition was changed
InitialAssessmentQuestions	Long Text	Questions; a mini-medical record of initial questions to ask and document for each caller
InitialAssessmentQuestions_Last	Date/Time	Last date that the Initial Assessment Questions were updated
Background	Long Text	Background information for this guideline: causes, types of injuries, general info, etc.
BackgroundXHTML	Long Text	Background information stored as XHTML
BackGround_LastUpdate	Date/Time	Last date that the Background was updated
FirstAid	Long Text	FirstAid information stored as TEXT
FirstAidXHTML	Long Text	First Aid Information stored in memo field as XHTML
FirstAid_LastUpdate	Date/Time	Last date that the First Aid was updated
Reference_LastUpdate	Date/Time	Last date that the References for this guideline were updated
SearchWords_LastUpdate	Date/Time	Last date that the Searchwords for this guideline were updated
Questions_LastUpdate	Date/Time	Last date that the Triage Questions for this guideline were updated
CA_LastUpdate	Date/Time	Last date that the Care Advice for this guideline were updated
AH_DESCRIPTOR	Yes/No	-----
Category	Short Text	Adult, Pediatric
Group	Short Text	After-Hours Version, etc.
Type	Short Text	Definition Type
System	Short Text	General, Gastrointestinal, etc.
Anatomy	Short Text	Abdomen, Head, etc.
VersionYear	Short Text	Version Year
Status	Short Text	Status information used by authors: Active, Inactive, Draft; ONLY ACTIVE guidelines should be used.
Acuity	Number	Numeric Scale, 1-5, 1 is most serious (most urgent)
Gender	Short Text	Gender at birth for this guideline: F, M, B
AgeGroup	Short Text	Protocol appropriate for Adult, Pediatric
Min_Age_Years	Number	Minimum age for this guideline in years
Max_Age_Years	Number	Maximum age for this guideline in years
Min_Age_Months	Number	Minimum age for this guideline in months
Max_Age_Months	Number	Maximum age for this guideline in months
WH	Yes/No	Women's Health
BH	Yes/No	Behavioral Health
OA	Yes/No	Older Adult Health
CD	Yes/No	Chronic Disease
Hospice	Yes/No	Hospice
Oncology	Yes/No	Oncology (Cancer)
Prescription_Option	Yes/No	Is there an option for prescription medication in the Care Advice?

For example, here is text from the *Algorithm.Definition* field for the *Constipation* guideline.

- * Infrequent bowel movements: Patient feels like bowel movements do not occur frequently enough.
- * Difficulty passing bowel movements: Patient reports straining, hard stools, or rectal pressure.
- * Fecal impaction (hard stool stuck in rectum): Symptoms may include rectal pain, rectal pressure, leakage of stool, back or abdominal pain. Patient may report that manual removal (disimpaction) or enemas are needed to pass stool.

Constipation SEVERITY is defined as:

- * NO CONSTIPATION (SCALE 0): Able to pass stool on a regular basis without discomfort.
- * MILD (SCALE 1-3): Intermittent symptoms; occasional use of dietary modifications (e.g., high fiber), stool softeners, laxatives, or enema.
- * MODERATE (SCALE 4-7): Frequent or ongoing symptoms; regular use of dietary modifications (e.g., high fiber), stool softeners, laxatives, or enema.
- * SEVERE (SCALE 8-10 OR 'WORST POSSIBLE'): Frequent, ongoing symptoms; manual evacuation or disimpaction required.

Note:

- * Associated symptoms can include abdominal bloating, cramping, malaise, and nausea.
- * Any abdomen pain from constipation is mild, intermittent (comes and goes), and cramping. Constant abdominal pain should be triaged using one of the abdominal pain guidelines.

Table – AlgorithmReference

This table provides a one-many-one relationship between **Algorithm** and **Reference** tables.

Field Name	Data Type	Description (Optional)
AlgorithmID	Number	Secondary ID
ReferenceID	Number	Secondary ID
DateUsed	Date/Time	Date reference was first used for this guideline
LastUpdate	Date/Time	Date updated

Table – AlgorithmSearchwords

This table provides a one-many-one relationship between **Algorithm** and **Searchword** tables.

Field Name	Data Type	Description (Optional)
AlgorithmID	Number	Secondary key
SearchWord	Short Text	Secondary key
CreateDate	Date/Time	Date this searchword was first linked to this guideline
LastUpdate	Date/Time	

Table – AlgorithmSupplemental

This table provides a one-many-one relationship between **Algorithm** and **Supplemental**.

Field Name	Data Type	Description (Optional)
AlgorithmID	Number	Secondary key
SupplementalID	Number	Secondary key
CreateDate	Date/Time	Date supplemental information first linked to this guideline

Table – Disposition

This table lists the Dispositions (e.g., EMS 911 Now, Go to ED Now), See PCP Within 24 Hours) to which each Triage Assessment Questions will map.

Field Name	Data Type	Description (Optional)
LevelID	Number	Unique ID
CreateDate	Date/Time	Date record created
LastUpdate	Date/Time	Last date that the record was changed
DispositionHeading	Short Text	Disposition Heading to use to group and separate triage questions by disposition level.
DispositionHeading_Telemedicine	Short Text	A parallel set of dispositions for clients, facilities, or healthcare systems that have / use telemedicine.
Adult_CareAdvice_Number	Number	Default Care Advice Number used for ADULT for this disposition level.
Adult_CareAdvice_Statement	Long Text	Default Care Advice Statement used for ADULT for this disposition level.
Adult_CareAdvice_Statement_XHTML	Long Text	Default Care Advice Statement used for ADULT for this disposition level formatted as XHTML.
Adult_CareAdvice_Statement_Telemedicine	Long Text	Default Care Advice Statement used for ADULT for this TELEMEDICINE disposition level.
Adult_CareAdvice_Statement_XHTML_Telemedicine	Long Text	Default Care Advice Statement used for ADULT for this TELEMEDICINE disposition level formatted as XHTML.
Pediatric_CareAdvice_Number	Number	Default Care Advice Number used for PEDS for this disposition level.
Pediatric_CareAdvice_Statement	Long Text	Default Care Advice Statement used for PEDS for this disposition level.
Pediatric_CareAdvice_Statement_XHTML	Long Text	Default Care Advice Statement used for PEDS for this disposition level formatted as XHTML.
Pediatric_CareAdvice_Statement_Telemedicine	Long Text	Default Care Advice Statement used for PEDS for this TELEMEDICINE disposition level.
Pediatric_CareAdvice_Statement_XHTML_Telemedicine	Long Text	Default Care Advice Statement used for PEDS for this TELEMEDICINE disposition level formatted as XHTML.

Table – Question

This table contains the Triage Assessment Questions (TAQs) for each guideline.

Field Name	Data Type	Description (Optional)
QuestionID	AutoNumber	Unique ID; primary key
AlgorithmID	Number	Secondary key; identifies which guideline uses this TAQ
QuestionOrder	Number	Order that questions appear within a specific disposition level
DateCreated	Date/Time	Date this TAQ was created
LastUpdate	Date/Time	Date this TAQ was last updated
Question	Long Text	The Triage Assessment Question (TAQ) as text
DispositionLevel	Number	The disposition level is used to group and sort triage questions by acuity (disposition level).
Information	Long Text	Information for the NURSE that explains the triage question is stored here (R/O, reasons, rationale, notes).
SMAG_LINK_ID	Number	Link to a more appropriate or specific guideline (See More Appropriate Guideline) using AlgorithmID from Algorithm Table
CMS_NEW	Yes/No	Field for Telemedicine Hand-Offs
TelemedicineEligible	Yes/No	YES: A positive response to this triage question (TAQ) usually can be evaluated and managed in a VIDEO telemedicine encounter

Here are several example TAQs from the Back Injury guideline with a disposition of Go to ED Now. Note that for most TAQs (*Question.Question*) there is also additional information (*Question.Information*) for the nurse triager.

Go to ED Now

[1] SEVERE PAIN in kidney area (flank) AND [2] follows direct blow to that site

R/O: renal injury

CA: 41, 1

Blood in urine (red, pink, or tea-colored)

R/O: renal injury

CA: 41, 1

[1] Unable to urinate (or only a few drops) > 4 hours AND [2] bladder feels very full (e.g., palpable bladder or strong urge to urinate)

Question.Question

R/O: cauda equina syndrome, conus medullaris syndrome

Question.Information

CA: 41, 80, 1

[1] Loss of bladder or bowel control (urine or bowel incontinence; wetting self, leaking stool) AND [2] new-onset

R/O: urinary retention with overflow incontinence, cauda equina syndrome, conus medullaris syndrome

CA: 41, 80, 1

Numbness (loss of sensation) in groin or rectal area

R/O: cauda equina syndrome, conus medullaris syndrome

CA: 41, 80, 1

Skin is split open or gaping (or length > 1/2 inch or 12 mm)

Reason: may need laceration repair (e.g., sutures)

CA: 41, 109, 127, 1

Table – QuestionAdvice

This table provides a one-many-one relationship between the **Question** table to the **Advice** table.

	Field Name	Data Type	Description
🔑	QuestionID	Number	
🔑	AdviceID	Number	
	LastUpdate	Date/Time	
	QuestionAdviceOrder	Number	Order that the advice appears under each question.

Table – References

This table contains the references (article citations) for each guideline. These references show the evidence based for a guideline.

	Field Name	Data Type	Description
🔑	ReferenceID	AutoNumber	Unique ID
	TopicReferenceID	Number	Unique Reference Number from MTI
	ReferenceTitle	Text	Title
	ReferenceSource	Text	Source
	ReferenceAuthor	Text	Author
	LastUpdate	Date/Time	Date last updated
	DateAdded	Date/Time	Date created
	MDConsultURL	Hyperlink	
	PMID	Text	Unique PUBMED ID
	PubMedURL	Hyperlink	PubMed URL for abstract and citation for article.
	PublicURL	Hyperlink	Public URL (if exists) for full text of article.

Table – Searchwords

This table contains all of the search words (metadata; key words).

	Field Name	Data Type	Description (Optional)
🔑	SearchWord	Short Text	
	DateCreated	Date/Time	

Table – Supplemental

This table contains supplemental non-guideline content (e.g., list of reviewers, OTC drug dosage tables)

	Field Name	Data Type	Description
🔑	SupplementalID	AutoNumber	Unique ID
	Author	Text	name of author
	Filename	Text	Filename used for HTML file that is created; If or when database exports to html
	CreateDate	Date/Time	Date created
	LastUpDate	Date/Time	Last date that this was revised-changed
	LastReviewDate	Date/Time	Last date that this content was reviewed
	Title	Text	Title
	Title_LastUpdate	Date/Time	Last date that the title was changed
	Content_XHTML	Memo	General/Background Information -- Information stored in memo field as XHTML
	Content	Memo	General/Background Information -- Information stored in memo field as TEXT
	Content_LastUpdate	Date/Time	
	Category	Text	Descriptor: Adult OTC Drug Dosage Table, Pediatric Dosage Table...
	Group	Text	Stores whether this is supplemental topic is associated with adult or pediatric content.
	Status	Text	Status information used by authors: Active, Inactive, Draft

Table – System

A look-up table that is used for sorting, selecting, and reporting on the guidelines.

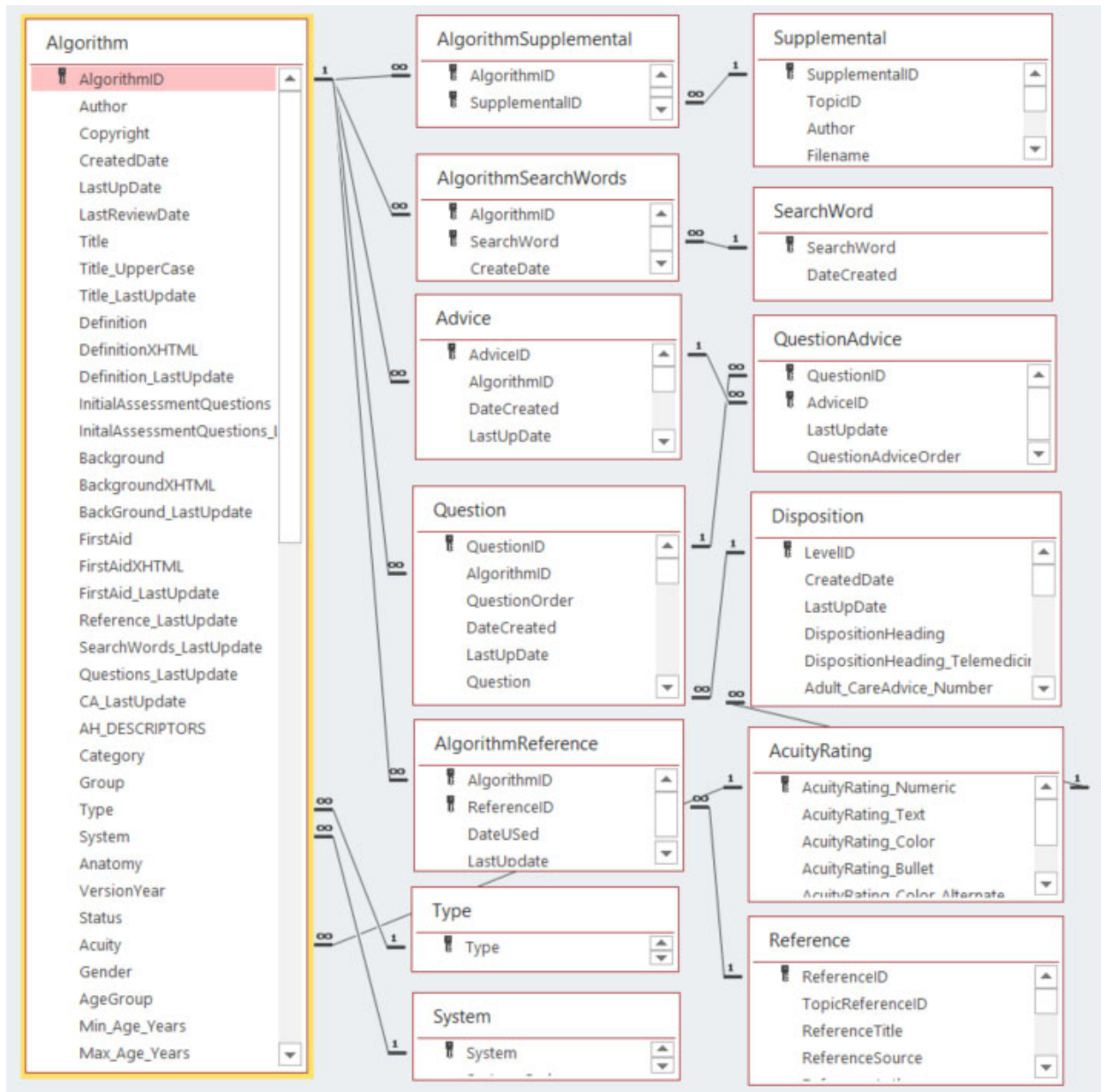
	Field Name	Data Type	Description
🔍	System	Text	
	System_Order	Number	
	System_Example	Text	

Table – Type

A look-up table that is used for sorting, selecting, and reporting on the guidelines.

	Field Name	Data Type	Description
🔍	Type	Text	
	Type_Topic	Text	

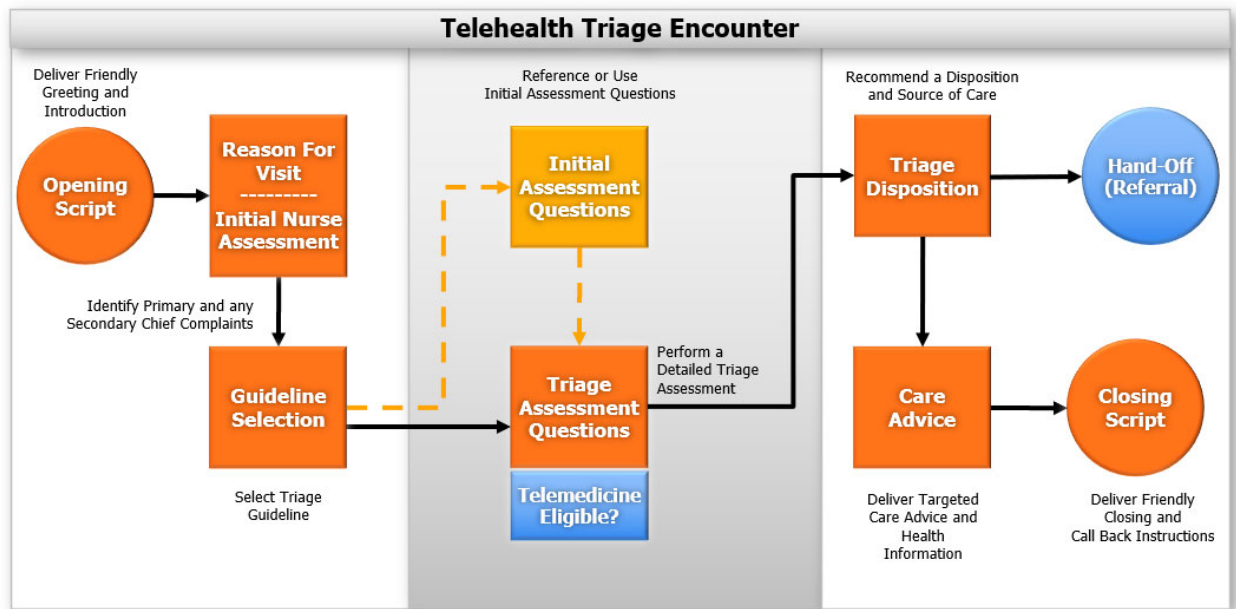
Database Relationship Diagram



Frequently Asked Questions

1. What is a typical process flow for a telehealth triage encounter?

Here is a swim lane flow chart showing a typical high-level flow for a telehealth triage encounter.



2. How long does a typical telehealth triage encounter between a triage nurse and a caller (e.g., patient, parent, caregiver) typically last?

An average call duration is 11 to 13 minutes.

There are many factors that affect the call duration. These include patient factors such as the chief complaint and age, nurse factors such as training and experience, and software user interface factors.

3. How might the Triage Assessment Questions appear in a telehealth triage software program?

Here is a sample user interface for a patient with a reason for visit (reason for call) of “ankle injury”.¹

Pathways Call Center Software

Patient

Profile

Encounter History

Triage Process

Reason for Call

Triage Questions

Care Advice

Encounter Summary

Resources

Background

First Aid

Triage Assessment Questions

Kathy Johnson RN

Patient Name: John Anderson Caller Name: Self Encounter Date: 10/3/2021
Age: 24 Relationship: Self
Gender at Birth: M PCP: Marcus Welby MD

Protocol: Ankle Injury (Adult)

☐

☐

Cast problems or questions
[Go to Guideline: Cast Symptoms and Questions \(Adult\)](#)

☐

☐

Go To ED Now

No To All

Yes

No

☐

☐

Bullet wound, stabbed by knife, or other serious penetrating wound
FIRST AID: If penetrating object still in place, don't remove it (Reason: removal could increase bleeding)

☐

☐

Skin is split open or gaping (or length > 1/2 inch or 12 mm)
Reason: may need laceration repair (e.g., sutures)

☐

☐

[1] Bleeding AND [2] won't stop after 10 minutes of direct pressure (using correct technique)
Reason: may need laceration repair (e.g., sutures)

☐

☐

[1] Dirt in the wound AND [2] not removed with 15 minutes of scrubbing
Reason: needs irrigation and/or additional wound care

☐

☐

Can't stand (bear weight) or walk
R/O: fracture

☐

☐

[1] Numbness (new loss of sensation) of toe(s) AND [2] present now

☐

☐

Sounds like a serious injury to the triager
R/O: fracture, dislocation

15

¹ Ask about the *Pathways Call Center* web demo to see more examples.

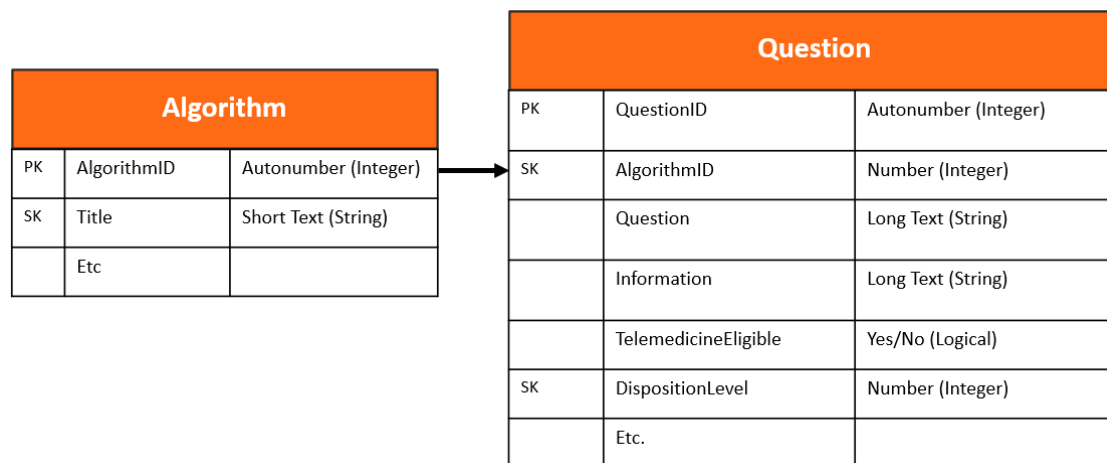
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4. How is telemedicine eligibility information stored in the database?

Each Triage Assessment Question (TAQ) record has a new associated field that stores whether the patient might be eligible for telemedicine care (telemedicine eligible).

Telemedicine eligible means that a patient with a positive response to this triage assessment question (TAQ) **usually can** be evaluated and managed in a video telemedicine encounter without referral to another site of care. Our STCC goal is to provide additional decision-support to telehealth nurses and identify patients that are eligible for safe telemedicine referral. This helps facilitate a more consistent hand-off from a nurse advice line or call center to a telemedicine provider.

The data for telemedicine eligibility is stored in the Question table in the logical (Yes/No) field called *Question.TelemedicineEligible*.



In a nurse-facing user interface, the value for *Question.TelemedicineEligible* could be presented in a number of different ways. For example, an icon can show whether the patient could be considered eligible for a video visit.

Video Visit	Yes - No	See Today in Office	Rationale
	☐ Yes ☒ No	[1] SEVERE pain (e.g., excruciating) AND [2] not improved 2 hours after pain medicine/ice packs,	R/O: fracture
	☒ Yes ☐ No	Suspicious history for the injury	R/O: domestic violence or elder abuse

We continue to recommend that your software provides call center leadership the ability to edit the Triage Assessment Questions and change the assigned Dispositions within the telehealth triage software.

We also recommend that you provide call center leadership the ability to edit the Telemedicine Eligible recommendations. Health systems and group practices may have different telemedicine capabilities and different practice policies for telemedicine referral.

Disposition Table

STCC has developed a set of parallel alternate Disposition headings for optional use when a call center or health organization refers patients to telemedicine as part of the telehealth triage process.

These **Telemedicine Disposition** headings are stored as a new column (field) in the Disposition table named *DispositionDispositionHeading_Telemedicine*.

Field Name	Data Type	Description (Optional)
LevelID	Number	Unique ID
CreatedDate	Date/Time	Date record created
LastUpDate	Date/Time	Last date that the record was changed
DispositionHeading	Short Text	Disposition Heading to use to group and separate triage questions by disposition level.
DispositionHeading_Telemedicine	Short Text	A parallel set of dispositions for clients, facilities, or healthcare systems that have / use telemedicine.
Adult_CareAdvice_Number	Number	Default Care Advice Number used for ADULT for this disposition level.

Here is a partial listing of the Disposition table showing the standard and telemedicine disposition headings side-by-side.

LevelID	DispositionHeading	DispositionHeading_Telemedicine	Video
100	Call EMS 911 Now	Call EMS 911 Now	
90	Go to ED Now	Go to ED Now	
85	Go to ED Now (or PCP triage)	Go to ED Now (or PCP triage)	
80	See HCP (or PCP Triage) Within 4 Hours	See HCP (or PCP triage or Video Visit) within 4 Hours	Yes
78	Call PCP Now	Call PCP or Video Visit Now	Yes
70	See PCP Within 24 Hours	See PCP or Video Visit Within 24 Hours	Yes
65	Call PCP Within 24 Hours	Call PCP or Video Visit Within 24 Hours	Yes
50	See PCP Within 3 Days	See PCP or Video Visit Within 3 Days	Yes
48	Call PCP When Office is Open	Call PCP or Video Visit When Office is Open	Yes
20	See PCP Within 2 Weeks	See PCP or Video Visit Within 2 Weeks	Yes
15	Home Care	Home Care	

5. Do you provide the content via SDK or API?

No. STCC does not provide an SDK nor deliver the content via API (e.g., web services).

STCC delivers the telehealth triage guideline content in an Access Database. Clients typically upsize this database to SQL Server (or other third party database) or import the data directly into their own software and proprietary database.

6. Can we put this database into another format?

Yes. Typically, you would build an import process into your own third party (e.g., MS SQL, MYSQL) or proprietary database or content management system.

7. How often is the content updated?

Every year STCC updates the telehealth triage guideline content and we send you a new database.

Sometimes more urgent updates are needed because of change in medical knowledge or other circumstances (e.g., COVID-19 pandemic). In such cases, we will send you an updated database.

8. Does STCC change the data model?

We do not change the data model on a routine basis so your import should work the same from year to year.

If we do make a change to the data model, we will provide you several months' advance notice of the change.

9. How are the larger chunks of text stored within the database?

Larger blocks of text such as the Definition, Background, First Aid, and Care Advice are stored as both XHTML and plain text in the database.

XHTML is preferred for displaying the content as it includes additional simple formatting (i.e., bold, italic, bullets, hyperlinks).

10. How are the Triage Assessment Questions (TAQs) presented and is the order important?

Yes. The order is important. The TAQs within each disposition level are presented in descending order of clinical acuity (severity) and importance.

Within your triage software you will group the TAQs by disposition level and sort by

- a. *Question.DispositionLevel* [Descending]
- b. *Question.QuestionOrder* [Ascending]

A SQL statement showing this might read as follows.

```
SELECT Question.AlgorithmID, Question.DispositionLevel, Question.QuestionOrder, Question.Question,
Question.Information
FROM Algorithm INNER JOIN Question ON Algorithm.AlgorithmID = Question.AlgorithmID
WHERE (((Question.AlgorithmID)=79))
ORDER BY Question.DispositionLevel DESC , Question.QuestionOrder;
```

11. Can I make changes to the Triage Assessment Questions (TAQs) or the Disposition Headings?

Yes, you are allowed to make changes to the disposition headings and TAQs in your software. This should be done under the direction of your clinical leadership.

Remember that STCC will be sending you an annual update. The changes that you have made to the guidelines will need to be reconciled in some manner to the additions, changes, and deletions made by STCC in the annual update.

12. Is there a way to show a list of guidelines in acuity order based on symptoms presented?

Yes, each guideline has an assigned acuity score of 1 to 5. Please review the **Acuity** table.

You can show a list of relevant guidelines to the triage nurse sorted by acuity, thus indicating which guideline is most serious.

13. How can we tell if a record has been in the database has been changed?

All of the tables have fields that show when the record was created and last changed. For example, here is the Question table.

	Field Name	Data Type	Description (Optional)
❗	QuestionID	AutoNumber	Unique ID; primary key
	AlgorithmID	Number	Secondary key; identifies which guideline uses this TAQ
	QuestionOrder	Number	Order that questions appear within a specific disposition level
	DateCreated	Date/Time	Date this TAQ was created
	LastUpDate	Date/Time	Date this TAQ was last updated
	Question	Long Text	The Triage Assessment Question (TAQ) as text
	DispositionLevel	Number	The disposition level is used to group and sort triage questions by acuity (disposition level).
	Information	Long Text	Information for the NURSE that explains the triage question is stored here (R/O, reasons, rationale, notes).
	SMAG_LINK_ID	Number	Link to a more appropriate or specific guideline (See More Appropriate Guideline) using AlgorithmID

14. Can we make changes to the content based on client need?

Some of our clients choose to make changes to the telehealth triage guidelines based on local healthcare needs and input from their clinical leadership.

Remember that STCC will be sending you an annual update. The changes that you have made to the guidelines will need to be reconciled in some manner to additions, changes, and deletions made by STCC in the annual update.

15. What is the purpose of the Reference table?

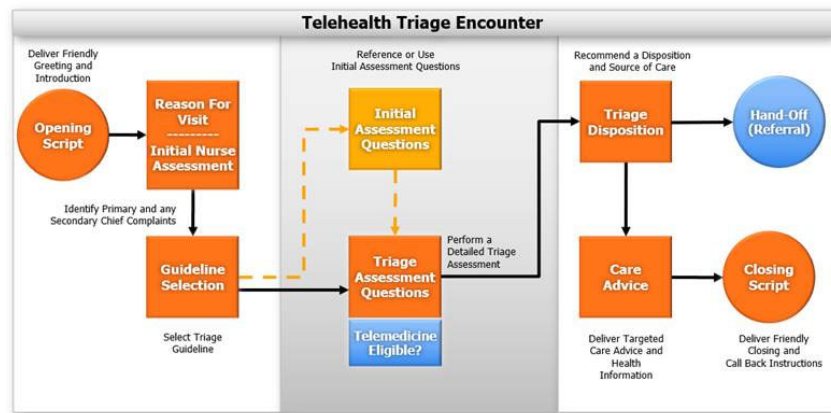
The Reference table stores the references that show the evidence-base for the decision logic and advice in the telehealth guidelines.

References are mostly used by the clinical leadership of a call center or healthcare organization.

The nurse triager will not use the references during a normal telehealth encounter with a patient.

16. Which Care Advice should a triager be able to select and send to a patient?

In a typical telehealth encounter the triager provides Care Advice at the end of the encounter after determining the triage disposition.



Broadly speaking, there are four types of Care Advice that a triager may share with a caller.

	Disposition Care Advice	Note to Triager Advice	General Care Advice	Call Back If Advice
Give Now to Patient (Verbally)	Yes	Yes	Yes	Yes
Send Later to Patient (e.g., Patient Portal, Encrypted Mobile App, Text Message)	No	No	Yes	Yes
Value of PatientHealthInfo field in Advice Table (Advice.PatientHealthInfo)	No	No	Yes	Yes

A triager should be allowed to select and send Care Advice that is either General Care Advice or Call Back If Care Advice. The Care Advice that a triager should be allowed to select and send is the same as the value of the *Advice.PatientHealthInfo* field.

Field Name	Data Type	Description (Optional)
AdviceID	AutoNumber	Unique ID
AlgorithmID	Number	
DateCreated	Date/Time	Date created
LastUpdate	Date/Time	Date last updated
LastUpdate_XHTML	Date/Time	
Advice	Long Text	Care advice statement in TEXT
Advice_XHTML	Long Text	Care advice statement in XHTML
PatientHealthInfo	Yes/No	Care advice appropriate for distribution to patient via web portal or email.
AdviceSnap	Short Text	First line of Care Advice Statement; used by Authors; not needed by end-users
AlgorithmOrder	Number	Used by Authors to track equivalent care advice

Here is a simplified user interface showing how a nurse might document Care Advice that they have selected to give now or send later.

Pathways Call Center Software

Patient

Profile

Encounter History

Triage Process

Reason for Call

Triage Questions

Care Advice

Encounter Summary

Resources

Background

First Aid

Initial Assessment Questions

Powered by:

Schmitt-Thompson Clinical Content

Care Advice

Kathy Johnson RN

Patient Name: John Anderson

Caller Name: Self

Encounter Date: 4/18/2023

Age: 24

Relationship: Self

Gender at Birth: M

PCP: Marcus Welby MD

Protocol: Ankle and Foot Injury (Adult)

Question: Large swelling or bruise(> 2 inches or 5 cm)

Given Now

Send Later

Apply a Cold Pack

- Apply a cold pack or an ice bag (wrapped in a moist towel) to the area for 20 minutes. Repeat in 1 hour, then every 4 hours while awake.
- Continue this for the first 48 hours after an injury.
- This will help decrease pain and swelling.

Pain Medicines

- For pain relief, you can take either acetaminophen, ibuprofen, or naproxen.
- They are over-the-counter (OTC) pain drugs. You can buy them at the drugstore.
- Acetaminophen - Regular Strength Tylenol:** Take 650 mg (two 325 mg pills) by mouth every 4-6 hours as needed. Each Regular Strength Tylenol pill has 325 mg of acetaminophen. The most you should take each day is 3,250 mg (10 pills a day).
- Acetaminophen - Extra Strength Tylenol:** Take 1,000 mg (two 500 mg pills) every 8 hours as needed. Each Extra Strength Tylenol pill has 500 mg of acetaminophen. The most you should take each day is 3,000 mg (6 pills a day).
- Ibuprofen (e.g., Motrin, Advil):** Take 400 mg (two 200 mg pills) by mouth every 6 hours. The most you should take each day is 1,200 mg (six 200 mg pills), unless your doctor has told you to take more.
- Naproxen (e.g., Aleve):** Take 220 mg (one 220 mg pill) by mouth every 8 hours as needed. You may take 440 mg (two 220 mg pills) for your first dose. The most you should take each day is 660 mg (three 220 mg pills a day), unless your doctor has told you to take more.

Pain Medicines - Extra Notes and Warnings

- Use the lowest amount of medicine that makes your pain better.
- Acetaminophen is thought to be safer than ibuprofen or naproxen in people over 65 years old.
- Acetaminophen is in many OTC and prescription medicines. It might be in more than one medicine that you are taking. You need to be careful and not take too much. An acetaminophen overdose can

BACK

DONE